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Effective Motivational Interviewing with Court-Mandated Clients Charged with Sexual Offenses
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Court-mandated therapy for individuals charged with sexual offenses often faces significant resistance from clients, who may feel alienated or marginalized by the criminal justice system. This resistance is especially pronounced because of societal stigma, public shaming, and the unique challenges these clients face while navigating legal supervision. Understanding and addressing this resistance is crucial for clinicians aiming to implement effective therapeutic interventions. One approach that has shown promise in engaging these clients is Motivational Interviewing (MI). MI is person-centered and collaborative, fostering engagement and encouraging change from within. This paper argues that MI, by reducing resistance and accessing change talk, can be a powerful tool for engaging court-mandated clients charged with sexual offenses, facilitating therapeutic progress, and ultimately reducing recidivism.

The impact of being charged with a sexual offense does not compare to the impact of being a victim of sexual violence. This paper does not aim to minimize, excuse, or to leave unaddressed the damage that these offenses cause. To be aware of this damage may seem contrary to treating clients with these charges. However, in her 2023 work *Truth and Repair*, respected author Judith Herman found that victims of sexual and domestic violence often seek to be heard, believed, and reassured that such violence will not happen again—to themselves or anyone else. For mental health clinicians truly committed to preventing further violence, effective treatment requires a deep and nuanced understanding of both the consequences of sexual violence and the shame, ostracizing, restriction, and trauma experience by individuals charged with sexual offenses.

Accessing Accurate Empathy in Motivational Interviewing with Court-Mandated Clients Charged with Sexual Offenses

To effectively utilize MI, Miller and Rollnick (2002), argue that accurate empathy is crucial. Based on Rogers' (1957) interpretation of necessary and sufficient conditions for change, accurate empathy requires skilled reflective listening that amplifies the client's own experience and meaning without imposing the clinician's own views. Because individuals convicted of sexual offenses are among the most judged category of criminally justice-involved populations (Craig, 2005), this can be especially difficult. Clinicians may struggle to access empathy without understanding the underlying processes of resistance, defensiveness, and the context of being convicted of a sexual offense.

Due to the shame associated with these crimes, clients may not be interested in self-identifying as a "sex offender". The label itself may be antithetical to present change narratives; Willis (2018) argues that not using this label in treatment may be important to desistance (the act of abstaining from criminal behaviors) processes for clients. In fact, if clients identify with being a sex offender, instead of as a person who did something that they wish they had never done, this may be a bad sign for commitment to change (Maruna, 2012). Similarly, if clinicians believe that their clients are and will always be "sex offenders", that message may influence treatment. If treatment can reduce reoffending, it follows that people who have committed sexual violence once are not predestined to repeat it.

Additionally, clinicians should understand that being convicted of a sexual offense often creates conditions correlated with criminogenic risk, including transient housing or homelessness, loss of social support, unemployment, and social rejection (Andrews & Bonta, 2017; Pettus-Davis et al., 2019; Uggen et al., 2004; Western et al., 2015). These conditions can contribute to further offending. Being arrested, charged, registered as a sex offender, and publicly identified can also be traumatic experiences.

Clients living under these conditions often report symptoms of PTSD. In a qualitative study, Harris and Levenson (2024) found that clients experienced rumination, shame, low self-worth, hypervigilance around parole violations, and suicidal ideation. One participant described intrusive thoughts: *“I could be having a good day and any little thing will remind me of my situation, and it’ll just suck the life out of that day and it becomes a hopeless, ruminating thought* (Harris & Levenson, 2024, p. 16).” Another described shame: *“That was very, very wrong. You feel accused, you feel ashamed, you feel guilty because you done things that you shouldn’t be proud of... You feel negative about yourself* (Harris & Levenson, 2024, p. 17).” Suicidal ideation was also common: *“My mind frame is: the next time I violate, if I have any chance whatsoever, I’m gonna kill myself. I’m not goin’ to jail for 15 years. What’s the point? ... not that I do anything wrong but what if I eat poppy seeds on a bagel or somethin’ and I had a positive drug test? I’d go find the nearest bridge or somethin’ like that. It would be the first idea that I have. I think about that a lot* (Harris & Levenson, 2024, p. 17).”

It is also important for clinicians to know that individuals convicted of sexual offenses have some of the lowest reoffending rates of any criminal population, and most recidivism is due to parole violations rather than new offenses. In New York State, the three-year reoffense rate is approximately 29%, with 25–28% due to parole violations (NYS, 2019). Increased scrutiny and supervision may contribute to these high violation rates. Although appropriate monitoring and treatment reduce reoffending (NYSDCJS, 2008), some parole officers may hold stigmatizing beliefs, leading to stricter supervision and increasing the risk of accidental violations.

The resulting fear and anxiety can be intense. As one participant stated, *“Everywhere you go, everything you do... you’re in this constant state of panic, constant state of paranoia...”* (Harris & Levenson, 2024, p. 18).

Some clients may receive judgement from their communities and social networks - harassment in public because crimes were made public or because the clients are on the sex offender registry. This added social “punishment” may increase defensive and non-introspective states, often resulting in a high amount of resistance when clinicians are trying to implement clinical interventions. Clients often feel traumatized by the criminal justice system, especially those charged with sex offenses, because of treatment by the criminal justice system. Parole officers, judges and court staff may also be judgemental, shaming and unkind. Clinicians may be seen as a greater part of this whole when pushing clients to explore accountability before establishing a therapeutic relationship or decreasing resistance by effectively utilizing MI.

Accessing Change Talk and Present Motivation in Court-Mandated Clients

Accessing change talk in this environment is challenging. MI does not force change; it works collaboratively to strengthen the client’s own existing motivations. It is not psychoeducation or a generic person-centered approach, nor does it attempt to impose external goals on the client (Miller & Rollnick, 2009). MI meets clients where they are—an essential stance for a population facing barriers such as shame, limited social support, and the stresses of criminal justice involvement.

Another barrier is that some clients may not fully recognize the harm they have caused. Some clients were victims of sexual abuse as children, and this may have been the only “positive” interaction they had with caregivers. Cognitive distortions related to sexual consent, boundaries, gender norms and sexual violence may be present. Some clients may believe that physical cues, dress, and social behavior indicate consent. These distortions may arise from relationship schema, misinterpretations of societal norms, media representation (such as illicit sexual material or pornography), and

physical sensations (all explored by Ward and Casey, 2010). Motivation for treatment may be low if the client does not believe that wrongdoing occurred. Additionally, mandated treatment itself generates resistance (Hohman et al., 2009; Storch & Lane, 1989).

Despite these barriers, MI can be a pivotal tool. It focuses on exploring and resolving ambivalence, strengthening self-efficacy, enhancing perceived discrepancy, and supporting emerging change narratives. Pinto de Silva et al. (2025) describe MI as high-quality reflective listening paired with a comprehensive communication style. Because many individuals charged with sexual offenses have significant histories of childhood trauma (Levenson et al., 2016), clinicians may be perceived as authority figures and met with automatic resistance. The non-forceful nature of MI is therefore particularly useful and aligns with trauma-informed care, which Levenson et al. (2016) recommend.

Clients under community supervision may also be coping with major lifestyle changes, reduced social networks, and common criminogenic needs such as difficulties with impulse control, emotional regulation, and social skills (an omnipresent trifecta in criminological populations). There may be *a lot* that they already want to change.

Mann et al. (2002), recognize that clinician labeling, shaming, or expectation of resistance may hinder efforts to identify existing motivation. Effective MI requires counselors to be curious, non-judgmental, highly attuned, and ready to explore ambivalence. Although MI with this population may feel like wading through resistance, real opportunities for positive change and reduced reoffending lie beneath.

Motivational Interviewing and Recidivism Impact

MI may reduce recidivism in part by increasing adherence to court-mandated programs. In their systematic review and meta-analysis, Pinto de Silva et al. (2025) found that MI improves attendance and reduces dropout. They also found a statistically significant relationship between MI and reduced recidivism. Other studies link MI to improved engagement, retention, and adherence (Hettema et al., 2005), as well as lower recidivism in sexual offending populations (Cunha et al., 2024; Mann et al., 2002; Pinto et al., 2005).

In a qualitative review, Levenson et al. (2024) found that MI is a key component of effective treatment when embedded within trauma-informed care and a strong therapeutic alliance. A study in New Zealand involving primarily Māori participants found increased motivation to change among high-risk offenders in short-term treatments (Austin et al., 2011). MI's collaborative and culturally adaptable nature may explain this result. This adaptability is especially important in a criminal justice system where people of color are disproportionately charged with crimes that are committed at equal rates across races (Stolzenberg et al., 2013), and where clinicians often differ from their clients in race and socioeconomic status (DataUSA, n.d.).

In conclusion, while working with court-mandated clients charged with sexual offenses presents unique challenges, MI offers a trauma-informed, client-centered approach that can significantly improve engagement and reduce resistance. By acknowledging the complexities of shame, trauma, and cognitive distortions within this population, clinicians can use MI to access existing motivation for change. This approach supports treatment adherence, reduces recidivism, and increases the likelihood of long-term success. As therapeutic modalities continue to evolve, the priority must remain on creating empathetic, nonjudgmental spaces that support reflection, growth, and desistance from harmful behaviors.

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